6.21 Hepatic Encephalopathy*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family report confusion and drowsiness **Objective** • Asterixis • Disoriented to time and place • Raised ammonia	**ACTUAL** Acute confusion related to ammonia accumulation as evidenced by disorientation and asterixis.	• Back to usual orientation within 72 hours.	• Give lactulose as prescribed, adjusted to 2–3 soft stools a day. • Give rifaximin if ordered. • Avoid sedatives. • Look for and treat triggers: GI bleeding, infection, constipation, electrolyte imbalance. • Assess orientation every shift.	• Considered met once he is oriented.
Subjective • Family say, “He forgets to eat.” **Objective** • Cannot feed himself • Poor attention	**ACTUAL** Decreased feeding abilities related to confusion as evidenced by inability to feed independently.	• Takes in enough food.	• Help him at mealtimes. • Do not restrict protein unless ordered. • Chart his intake.	• Goal reached when intake is adequate.
Subjective *No specific complaint* **Objective** • Confused • Weak	**RISK** Risk for adult falls as evidenced by risk factors: confusion and weakness.	• No falls.	• Put fall precautions in place. • Keep the bed low and the call bell within reach. • Help him walk.	• Met if he does not fall.

*References for this care plan: [32], [35], [75], [129]. Full entries are listed in the References section of the CPMN book.